

Pain Management

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Introduction

Pain is a primary reason people seek healthcare and is associated with increased length of hospital stay, longer recovery time, and poorer patient outcomes. Pain is a multidimensional experience, with a variety of physiologic and psychosocial elements that can affect all aspects of an individual's life. The discomfort and distress associated with pain can last beyond the tissue-damaging experience.⁽¹⁾ Inadequate pain assessment can occur because of the patient's inability to express the occurrence of pain, negative impacts of expressing pain (eg, fear of receiving an injection, having more tests or medical treatments, or giving up independence), or because the patient's culture prevents pain expression (eg, culture places importance on enduring pain stoically or views pain expression as a sign of weakness).⁽¹⁾⁽²⁾⁽³⁾ Understanding of beliefs and meanings that drive pain behaviors is important to improve outcomes.⁽³⁾ Development of a personalized pain-management treatment plan can address the elements of pain affecting the patient's life.⁽⁴⁾

This tool is an overview of the assessment, management, and evaluation of pain. It includes management considerations for special populations or conditions.

Pain Management

Nursing Management

- Nursing pain-management process includes:
 - **Assessment**⁽⁵⁾
 - Patient's age, cognitive ability, communication ability, and health literacy
 - Provider and patient interaction
 - Provider and patient relationship
 - **Planning** should include⁽⁶⁾:
 - Comorbidities
 - Types of pain
 - Interventions
 - Medications (prescription and over-the-counter)
 - Nonpharmacologic therapies (eg, massage, positioning, relaxation)
 - Reassessment at appropriate intervals to determine effectiveness
 - Patient input regarding type of medication used
 - **Interventions** may include^[A]:
 - Review provider's notes to determine goal, methods, and medication dosages for pain management.
 - Check vital signs and level of consciousness.
 - Note amount of medication used.
 - Remain with patient to determine effectiveness, and watch for adverse reactions.
 - Assess patient's response to treatment and self-care.
 - Establish and implement education plan.
 - Information enables a patient to feel more in control of the situation when pain arises, which modifies the perception of pain.

- When possible, involve family in the education process so that family members will be informed and supportive of the patient's decisions.
- Document pain qualities, responses, medication, relief techniques, and corresponding patient or caregiver education.
- **Evaluation** of pain and response to treatment:
 - Regular intervals
 - Presence of unrelieved pain
 - Efficacy of treatments
 - Full relief
 - Moderate relief
 - No relief

Pain Management Assessment & Evaluation Tool

- **Pain assessment interval**
 - Initial clinical assessment
 - After pain-producing procedure
 - New report of pain
 - With vital signs or at set interval(1)
- **Pain history(1)**
 - Drug history
 - Drug allergies
 - Name of drug: _____
 - Name of drug: _____
 - Name of drug: _____
 - Drug intolerance
 - Name of drug: _____
 - Name of drug: _____
 - Name of drug: _____
 - Medical conditions that could affect pain(7):
 - Arthritis
 - Cancer
 - Depression, anxiety, or thought disorder
 - Edema
 - Fibromyalgia
 - Infection
 - Migraine
 - Neuropathy
 - Orthopedic condition
 - Peripheral vascular disease (PVD)
 - Surgical intervention
 - Wound complications (eg, dehiscence)
 - Other: _____
 - Treatments that cause pain
 - Dressing changes
 - Rehabilitation therapy (eg, PT)
 - Other: _____
 - Previous methods used for pain management and their efficacy
 - Over-the-counter medicines
 - 1. Type of medicine used: _____
 - Dose: _____
 - Frequency: _____
 - Duration: _____
 - Maximum dose used
 - Full relief
 - Moderate relief
 - No relief
 - Occasional dose used
 - Full relief
 - Moderate relief
 - No relief
 - 2. Type of medicine used: _____
 - Dose: _____
 - Frequency: _____

- Duration: _____
 - Maximum dose used
 - Full relief
 - Moderate relief
 - No relief
 - Occasional dose used
 - Full relief
 - Moderate relief
 - No relief
- 3. Type of medicine used: _____
 - Dose: _____
 - Frequency: _____
 - Duration: _____
 - Maximum dose used
 - Full relief
 - Moderate relief
 - No relief
 - Occasional dose used
 - Full relief
 - Moderate relief
 - No relief
- Prescription medicines (eg, opioids)
 - 1. Type of medicine used: _____
 - Dose: _____
 - Frequency: _____
 - Duration: _____
 - Maximum dose used
 - Full relief
 - Moderate relief
 - No relief
 - Occasional dose used
 - Full relief
 - Moderate relief
 - No relief
 - 2. Type of medicine used: _____
 - Dose: _____
 - Frequency: _____
 - Duration: _____
 - Maximum dose used
 - Full relief
 - Moderate relief
 - No relief
 - Occasional dose used
 - Full relief
 - Moderate relief
 - No relief
 - 3. Type of medicine used: _____
 - Dose: _____
 - Frequency: _____
 - Duration: _____
 - Maximum dose used
 - Full relief
 - Moderate relief
 - No relief
 - Occasional dose used
 - Full relief
 - Moderate relief
 - No relief
- Drug therapy complications
 - Overdose(1)
 - Need for increased dosage to achieve pain relief
 - Constant euphoria rather than pain relief

- Side effects
 - Constipation(8)
 - Nausea
 - Vomiting
 - Anorexia
 - Confusion
 - Sleep disturbance
 - Oversedation
 - Other: _____
- Other: _____
- Nonpharmacologic therapeutic interventions(1)
 - Massage
 - Application of heat (eg, heating pad)
 - Application of cold (eg, ice packs)
 - Acupuncture
 - Distraction techniques (eg, music, reading)
 - Positioning(9)
 - Relaxation techniques (eg, prayer, meditation)
 - Other: _____
- History of substance misuse[B][C]
 - Alcohol
 - Amphetamines
 - Barbiturates
 - Cocaine
 - Heroin
 - Marijuana
 - Nicotine
 - Other: _____
- Ways of expressing pain
 - Emotional response (eg, crying, irritability)
 - Vocal expression (eg, moaning)
 - Grimacing
 - Immobility
 - Other: _____
- Typical coping response to pain and stress
 - "Deals" with pain
 - Seeks relief
 - Other: _____
- Family's expectations and beliefs, which may include:
 - Pain should be completely relieved.
 - Some pain should be tolerated.
 - Pain medications may lead to addiction.
 - Other: _____
- **Quantifiable pain assessment(5)**
 - Intensity (using scale appropriate for optimal patient reporting, such as visual scale for patients with health literacy or language challenges)(7):
 - Number scale
 - 0 - no pain
 - 1
 - 2
 - 3
 - 4
 - 5 - moderate pain
 - 6
 - 7
 - 8
 - 9
 - 10 - extreme pain
 - Description scale
 - None
 - Just noticeable
 - Mild

- Moderate
- Tolerable
- Strong
- Severe
- Excruciating
- Intolerable
- Location
 - Head, specify location: _____
 - Neck, specify location: _____
 - Shoulder(s), specify location: _____
 - Arm(s), specify location: _____
 - Elbow(s), specify location: _____
 - Hand(s), specify location: _____
 - Finger(s), specify location: _____
 - Chest, specify location: _____
 - Abdomen, specify location: _____
 - Pelvis, specify location: _____
 - Leg(s), specify location: _____
 - Knee(s), specify location: _____
 - Foot/feet, specify location: _____
 - Toe(s), specify location: _____
 - Other: _____
- Quality (patient should describe nature or characteristics of pain):
 - Aching
 - Burning
 - Cold
 - Deep
 - Exhausting
 - Gnawing
 - Itchy
 - Nagging
 - Numb
 - Penetrating
 - Radiates
 - Sharp
 - Shooting
 - Stabbing
 - Tender
 - Throbbing
 - Tingling
 - Unbearable
 - Other: _____
- Duration:
 - Constant
 - Occasional
 - Intermittent
 - Specific time of day, specify: _____
 - Other: _____
- Precipitating factors:
 - Application of cold
 - Application of heat
 - Bowel movements
 - Eating
 - Exercise
 - Movement
 - Swallowing
 - Urinating
 - Weather (eg, rain)
 - Other: _____
- **Effects of pain(5)**
 - Behavior
 - Aggressive

- Combative
 - Depression
 - Disruptive
 - Irritable
 - Resists care
 - Wandering
 - Withdrawn
 - Body movements
 - Biting
 - Bracing
 - Guarding
 - Posture changes
 - Rigid
 - Breathing changes (eg, noisy, labored)
 - Change in appetite (eg, anorexia, comfort eating)
 - Constipation
 - Decreased physical activity
 - Diminished concentration
 - Exhaustion or tiredness
 - Facial expression (eg, grimace, frown, tear), specify:
-
- Inconsolable (inability to be consoled or distracted)
 - Insomnia
 - Nausea or vomiting
 - Sweating
 - Verbalization (eg, moaning, groaning, calling out)
 - Other: _____

Patient, Family, and Caregiver Education

- Relationship between patient's particular situation (eg, surgery, procedure, disease process) and pain(1):
 - Quality of pain:
 - Intensity
 - Duration
 - Location
 - Description (eg, dull, achy)
 - Relationship between pain and other activities (eg, coughing, ambulation, sleeping)
 - Techniques for modifying physical effects of pain
 - Application of heat (eg, heating pad)
 - Application of cold (eg, ice packs)
 - Elevation (eg, elevate affected limb, head of bed)
 - Massage
 - Splinting
 - Positioning(9)
 - Relaxation, cognitive techniques (eg, guided imagery), and stress management
- Pain assessment techniques(5)
 - Show or describe to patient the pain rating assessment that will be used. Explain each level on scale and its meaning.
 - Explain how often assessment will be done.
 - Set "trigger" level with patient.
 - Ask patient to report pain whenever it is above this level.
 - Inform patient that preemptive medication, or medication taken when pain is manageable, is more effective than taking medication after pain has become more severe.
- Promote patient's and carers' active participation in pain-management plan.
 - Goals for pain management
 - Side effects, adverse effects, and complications of pain-management techniques
 - Ability to manage pain plan (eg, safely manage medications, physical ability to position)
 - Relationship between current health situation and perception of pain (eg, cancer pain may have different meaning than pain during procedure)
- Medication types and timing and route of administration available to patient
 - Effects and side effects of possible medication and previous history or response to medications
 - Timing of administration (eg, as needed, around the clock)
 - Discuss PCA, epidural, or oral medication, as appropriate.
 - How to administer medication

- How to care for access site and equipment
 - How to prevent constipation (eg, use of stool softeners)
- Answer questions about possible addiction, dependence, unpleasant side effects, and other concerns about taking medication.
- Cognitive and behavioral coping techniques(7)
 - Use of coping techniques before onset of pain when possible
 - Relaxation exercises, which may include:
 - Deep, slow, relaxation breathing
 - Progressive relaxation
 - Focusing
 - Repeating soothing phrase mentally
 - Praying
 - Imagery
 - Meditation
 - Hypnosis(13)
 - Humor
 - Therapeutic touch
 - Art therapy
 - Music
- Nonpharmacologic pain-management techniques (eg, massage, positioning, use of ice/heat)
- When to seek medical intervention related to pain-management plan, such as:
 - Pain increases despite treatment.
 - Pain pattern is new or changes.
 - Medication side effects
 - Breathing pattern changes
 - Tachycardia
 - Inefficacy
 - Cognitive changes
 - Constipation

Pain Management for Special Populations

Children

- Considerations, information, and education(2)
 - Child should get age and developmentally appropriate preoperative or preprocedural pain education, with information about how the pain will be managed.
 - Understanding of emotional distress impact on child (due to lack of understanding about reason for procedure, or lack of active participation in informed consent), which can accentuate pain experience
 - Importance of supporting pain expression, emotional needs, and pain-management needs by treatment team
 - Educate parents and caregiver about pain-management options, and how and when to ask for pain relief for the child.
- Assessment of pain in children includes(2)(5):
 - By about 18 months of age, a child can begin to self-report pain. As the child matures, the amount of detail about intensity and quality included in the description of pain increases.
 - Observational measures when self-report is not available (these may underestimate child's pain but may give cues on pain level or efficacy of relief), including:
 - Facial expression
 - Physiologic changes
 - Vocalization
 - Crying
 - Sleeplessness
 - Activity
 - Consolability
 - Parents' observational ratings (may still underestimate pain level)
 - Rating scales and assessment tools for children (eg, Faces Pain Scale, which consists of 7 faces with expressions of increasing grimace. Child picks face that corresponds to their intensity). Use version in patient's native language.
- Pain-management interventions may include(2):
 - Pharmacologic interventions:
 - Acetaminophen or NSAIDs are the most common oral nonopioid analgesics.
 - Ketorolac is the only NSAID available in the United States that can be administered by IV and used postoperatively in pediatric patients.
 - Opioids, when indicated
 - For child in moderate to severe pain, either orally, subcutaneously, or through PCA
 - For infant younger than 2 months of age:
 - Administer initially in monitored setting.

- May cause increased respiratory depression and seizures, even when administered in doses that are safe to use in older children
- Nonpharmacologic therapeutic interventions (eg, massage, positioning, relaxation), with information on:
 - Desensitization is the process of gradually introducing anxiety-producing stimulus and allowing child to adapt to stimulus.
 - Positive reinforcement is a reward or acknowledgment of appropriate behavior.
 - Relaxation is a state of low arousal and comfort.(14)(15)(16)
 - Preparation is presentation of appropriate information in a timely and age-appropriate manner.
 - Imagery is mental pictures or a mental status that may counteract negative aspects or enhance positive aspects of the situation.
 - Hypnosis is altered state of consciousness characterized by focused attention and heightened responsiveness to suggestion.
 - Distraction is diversion of attention from anxiety-producing stimulus by providing another area of focus.
 - Modeling is showing patient desired behavior.
 - Rehearsal is giving the patient an opportunity to practice desired behavior.

Oncology Patients

- Considerations, information, and education
 - Cancer patients may have acute and chronic pain, and pain may change over time.
 - Pain changes may indicate need for further evaluation, diagnostic testing, or additional treatment.
- Pain-management interventions may include:
 - Pharmacologic interventions:
 - World Health Organization Pain Relief Ladder for mild, moderate, and severe cancer-related pain(17)
 - Step 1: Nonopioid analgesic, with or without adjuvant
 - Step 2: Mild opioid for mild to moderate pain, possible nonopioid, and possible adjuvant (if pain persists or increases)
 - Step 3: Strong opioid for moderate to severe pain, possible nonopioid, and possible adjuvant
 - Pain medication may be planned on regular schedule rather than on as-needed basis.
 - Manage possible side effects of pain medication.
 - Provide patient and caregiver with written pain-management plan.
 - Nonpharmacologic methods of management
 - Nonpharmacologic therapeutic interventions (eg, massage, positioning, relaxation)(1)
 - Psychological interventions include:
 - Referral to psychologist
 - Cognitive techniques focusing on perception and thought
 - Behavioral techniques to develop coping skills
 - Emotional, religious, or spiritual support

Elderly Patients and Patients with Cognitive Dysfunction

- Considerations, information, and education(18):
 - Increase risk of pain due to chronic condition (eg, osteoarthritis, diabetic neuropathy)
 - Untreated and undertreated pain may be associated with cognitive dysfunction, depression, social isolation, immobility, and sleep disturbances.(19)
 - Assessment considerations
 - Pain assessment may be complicated by dementia, impaired memory, reasoning, recognition, and communication deficits
 - Pain rating scales may be used reliably for patients without cognitive impairments.
 - Conduct assessment in quiet area with sufficient time to complete tool to decrease distractions.
 - Evaluate observational measures of indirect pain manifestations (eg, restlessness, agitation).
 - Vital sign deviation may not be apparent due to age-related diminished physiologic response.(20)
 - Provide ongoing assessment and guidance for appropriate use of exercise and activity. Promote balance between rest and exercise to maintain peak functional status and pain management.
- Pain-management interventions may include:
 - Pharmacologic methods of pain management(5)
 - Address polypharmacy when assessing pain and administering medication.
 - Drug-related problems, including drug-drug interactions, are higher in older adults, and patients taking more than 5 medications are more likely to receive an inappropriate prescription.
 - Risk factors for drug-related problems include an increased number of providers treating the patient, and concomitant use of drugs that cause additive hypotension, sedation, or anticholinergic effects.
 - Age-related changes in drug metabolism increase risk of serious side effects.
 - NSAIDs(1)
 - Effective for short-term pain management in the elderly but are associated with an increased risk of gastrointestinal bleeding episodes and adverse cardiovascular and cerebrovascular events. Proton pump inhibitor (PPI) should be used to reduce incidence of gastrointestinal bleeding.
 - Avoid in high doses and in patients with abnormal renal function or history of ulcer disease or bleeding.
 - NSAIDs should not be taken for long periods of time.

- World Health Organization (WHO) recommends NSAID usage in stepped approach (Step 1) for cancer patients in mild to moderate pain.
- Opioids are metabolized more slowly in older adults. Start medication at lower doses and titrate carefully.
- PCA can be used effectively in the elderly, but with greater risk of oversedation and respiratory depression due to drug accumulation.
 - Monitor closely for side effects, especially confusion, constipation, and sedation increasing risk for falls.
 - Different medication or adjunctive treatment should be used when side effects become intolerable.
- Nonpharmacologic therapeutic interventions (eg, massage, positioning, relaxation)(1)

Patients with Chronic Pain

- Considerations, information, and education
 - Chronic pain is pain that persists beyond the normal time frame despite condition or injury treatment and is not related to cancer. Absence of known physical, medical, or surgical causes of pain does not negate presence of chronic pain.(1)
 - Presentation includes:
 - Chronic benign pain persists for more than 6 months but does not necessarily change over time (eg, low back pain).
 - Recurrent acute pain associated with chronic conditions, with episodes that occur frequently over a period longer than 6 months and potentially over a lifetime (eg, migraine and trigeminal neuralgia)
 - Chronic progressive pain originates from a constant cause and is likely to increase in severity (eg, arthritis, migraines).
 - Physical manifestations (ie, tachycardia or overt pain behavior) may not present the same as in a patient with acute pain due to adaptation to pain over time.
 - Inadequate pain relief with pain-relieving interventions(1)
 - Multidisciplinary multimodal approach for effective management, including(1)(21):
 - Goal is to enhance ability to self-manage and modulate pain and to improve function with the pain that may persist.(22)
 - Comprehensive pain history assessment (similar to history for acute pain)
 - Physical examination: directed to neurologic, musculoskeletal, and other systems as appropriate
 - Psychosocial factors evaluated for presence of anxiety, depression, psychiatric disorders, coping mechanisms, family support or issues, and vocational and legal issues
 - Treatment plan that may include medication, PT or rehabilitation, stress management or counseling, group support, and vocational counseling
- Pain-management interventions may include[D]:
 - Nonpharmacologic therapeutic interventions (eg, massage, positioning, relaxation) and may also include(1):
 - Cognitive behavioral therapy
 - Pain-management programs
 - Support group participation(14)
 - Pharmacologic methods of pain management(15)
 - NSAIDs
 - Opioids
 - Antidepressants
 - Anticonvulsants
 - Topical agents (eg, diclofenac, lidocaine, capsaicin)
 - Invasive pain-management techniques(2)
 - Trigger point injections
 - Peripheral nerve blocks
 - Facet joint injections
 - Sympathetic blocks
 - Advanced neuromodulation(7)(23)
 - Spinal cord stimulation
 - Intrathecal drug administration

Patients Undergoing Surgery

- Preoperative plan(24)
 - Formulate plan for aggressive pain management with each patient, which includes the following factors:
 - Anticipated surgery (eg, laparoscopic vs open procedure)
 - Underlying medical conditions
 - Input from patient and surgeon
 - Capabilities of institution and specific anesthesiologist
 - Inform patient of pain options, including pharmacologic, mechanical, or behavioral modalities.
- Intraoperative management: optimized to reduce postoperative pain
- Postoperative pharmacologic pain management may include(5)(25):
 - PCA
 - Usually started in postanesthesia care unit (PACU) when patient is alert and cooperative.

- Most common PCA-administered narcotics are morphine, fentanyl, and hydromorphone. Anxiolytics may be coadministered as well.
- Allows individual titration that can be responsive to changing needs (eg, prior to ambulating or physical therapy)
- Permits constant, continuous therapeutic level of analgesic to avoid overmedicating or undermedicating, while providing the patient psychosocial manifestation of being in control of the management of one's own pain
- Indwelling catheters (eg, epidural, interpleural, intraplexus) with instillation of opiates, with or without local anesthetics
 - Epidural catheters should be located close to affected dermatome to decrease adverse effects when giving either local anesthetics or highly lipid-soluble narcotics. Common drugs used epidurally include morphine, fentanyl, hydromorphone, sufentanil, and bupivacaine, a local anesthetic.
 - Considerations for role and timing if concurrent use of IV heparin
 - Follow guidelines for using epidural or other catheters (eg, evaluate for migration, when to discontinue, complication monitoring).
- NSAIDs(7)
 - Advantage of NSAIDs because they work at distal sites different from those targeted by opiates or local anesthetics
 - Supply needed analgesia without respiratory depression
 - May be used preemptively, intraoperatively, and postoperatively (bleeding risk evaluation should be done prior to ordering postoperatively)
- Postoperative pain-management discharge plan should include(25):
 - Presence of caregiver to assist patient
 - Scheduled postoperative phone call to evaluate for care needs or issues
 - Provide 24-hour contact phone number.
 - Analgesia plan with instructions on administration, side effects, and drug and food interactions
 - Information to take pain medication before pain is severe

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Footnotes

[A] Organizational procedures should be provided and followed for pain management. [A in Context Link 1]

[B] A patient with a history of substance misuse may require larger doses of opioid medication to achieve relief.(10) A patient who is actively misusing alcohol may require adjunctive benzodiazepines to minimize anxiety and prevent complications of withdrawal.(11) [B in Context Link 1]

[C] Consider an addiction specialist, pain-management specialist, or psychiatrist referral, if indicated.(12) [C in Context Link 1]

[D] **National guidelines:** The Centers for Disease Control and Prevention (CDC) recommends nonpharmacologic and nonopioid therapy before opioids for management of chronic pain (not due to malignancy). Considerations for opioids should include pain-management goal setting and determining if benefits outweigh risks. When opioids are prescribed, they should be provided in conjunction with nonpharmacologic pain-management interventions and planned monitoring for misuse or use escalation.(21) [D in Context Link 1]

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